

Start or change contraception

STEP 1. Help the patient decide which method is the best option according to her needs.

- If wanting long term protection, consider IUD (5 years), subdermal implant (3-5 years), injectable (3 months), sterilisation (permanent).
- If needing quick return to fertility, consider IUD or subdermal implant.
- If worried about adherence issues, consider IUD, subdermal implant or injectable.
- If problems with heavy/painful/irregular periods, acne or premenstrual syndrome, consider COC.
- If the patient prefers to avoid hormones, consider copper IUD and/or reliable condom use (both hormone free), sterilisation or LNG-IUD (low dose hormone released locally into uterus).

STEP 2. Check if reasons to avoid chosen method (use table). If there is a reason, consider another method (or sterilisation, if appropriate) that will be acceptable to patient:

Heavy or painful periods
Use LNG-IUD or COC.

≥ 35 years old and smoker
Use IUD, implant, injectable or POP.

Medications

- If on rifampicin: use injectable, IUDs.
- If on phenytoin, carbamazepine: use injectable, IUD.
- If on lamotrigine: use IUD, implant, injectable, POP.
- If on EFV: use IUD or injectable.
- If on NVP, LPVr, ATVr: use IUD, implant or injectable.
- If on DTG: use IUD, implant, injectable, COC, POP.

Chronic conditions

- If breast cancer (avoid pregnancy for 5 years after diagnosis): use copper IUD.
- If cancer of uterus/cervix/ovary: use implant, injectable, COC, POP.
- If severe liver disease: use copper IUD.
- If history of blood clots: if stable on blood thinner, use implant, LNG-IUD, POP, injectable.
- If history stroke/TIA, heart attack, ischaemic heart disease: use IUD, implant, POP.
- If hypertension or BP ≥ 140/90, use: IUD, implant, POP. Only use injectable if BP < 160/100.
- If diabetes complications (eye, nerve, kidney damage): use IUD, implant, POP.

STEP 3. Explain possible side effects. If unacceptable to patient, consider another method.

STEP 4. Explain instructions for use (use table) and check understanding.

Method	Reasons to avoid	Side effects	Instructions for use
Intrauterine devices (IUDs) <i>(Small device fitted inside the uterus)</i> 1. Levonorgestrel IUD 52mg (LNG-IUD) 2. Copper IUD eg. Cu T380A	• Avoid both IUDs if: current STI/PID, unexplained vaginal bleeding, abnormality or cancer of cervix/uterus, or if unwell with advanced stage 3 or 4 HIV disease. • Also avoid LNG-IUD if: severe liver disease, breast cancer. • Also avoid copper IUD if: heavy/prolonged periods. • Postpartum (≤ 48 hours): avoid if chorioamnionitis, rupture of membranes for > 18 hours or postpartum haemorrhage.	• Discomfort or cramping during/ following insertion. • Menstrual abnormalities. • LNG-IUD usually results in no/ lighter periods, but may cause irregular bleeding. • Rarely, headaches with LNG-IUD.	• Trained staff to insert/remove. Insert any time during cycle. • If pain, give ibuprofen¹ 400mg 8 hourly with food for 3 days. • Gives long-term protection: 5 years. • No significant drug interactions expected. • Advise to return if excessive bleeding/pain, fever, foul-smelling discharge: refer. • If after delivery/miscarriage/TOP: insert ≤ 48 hours (if no reason to avoid) or ≥ 4 weeks.
Subdermal implant <i>(Small plastic rod/s just under skin of upper arm)</i> 1. Etonorgestrel 68mg (1x rod: 3 years) 2. Levonorgestrel 2x 75mg (2x rods: 5 years)	• Unexplained vaginal bleeding, previous breast cancer, liver disease. • Patient on rifampicin, efavirenz, phenytoin, carbamazepine.	• Pain/bruising. • Irregular bleeding, breast tenderness, weight gain, acne, headaches, moodiness, nausea.	• Trained staff to insert/remove. • If inserted after day 7 of cycle, use condoms/ abstain for 7 days. • If pain: give ibuprofen¹ 400mg 8 hourly with food for 3 days. • Gives long-term protection: 3-5 years depending on device used.
Injectable <i>(Long-lasting injection into upper arm)</i> eg. Medroxyprogesterone (DMPA) IM 150mg 12 weekly	Unexplained vaginal bleeding, breast cancer, ischaemic heart disease, stroke, severe liver disease, diabetes complications (eye, nerve, kidney damage).	Irregular, heavy, prolonged bleeding or no periods, hot flushes, breast tenderness, appetite changes, weight gain, acne, nausea/bloating.	• If started after day 7 of cycle, use condoms/ abstain for 7 days. • Protection lasts 3 months. • No need to adjust dosing interval for ART, TB or epilepsy treatment. • May be a delay in return of fertility (± 9 months).
Oral pill <i>(tablet to be swallowed every day)</i> 1. Combined oral contraceptive (COC): 1 tablet daily • Monophasic: eg. ethinylestradiol/levonorgestrel 30mcg/150mcg • Triphasic: eg. ethinylestradiol/levonorgestrel (varying doses) 2. Progestogen-only pill (POP): 1 tablet daily eg. levonorgestrel 30mcg	• Avoid both POP and COC if: breast cancer, severe liver disease or on rifampicin, phenytoin, carbamazepine, EFV, NVP, LPVr, ATVr. • Also avoid COC if: on lamotrigine, blood clots or stroke, smoker ≥ 35 years, migraines and ≥ 35 years or visual disturbances, BP ≥ 140/90, hypertension, CVD risk > 10%, ischaemic heart disease, diabetes complications (eye, nerve, kidney damage), for 6 weeks after delivery and for 6 months if breastfeeding.	Menstrual abnormalities, breast tenderness, headaches, moodiness, weight gain.	• If POP started after day 5 of cycle, use condoms/abstain for 2 days. If COC started after day 5, use condoms/abstain for 7 days. • Strict adherence needed: take every day at same time. POP less effective if taken ≥ 3 hours late. • If vomits < 2 hours or severe diarrhoea < 12 hours of taking pill, repeat dose. If > 24 hours diarrhoea/vomiting, use condoms or abstain. Continue for 7 days after better. • May be a delay in return of fertility (± 3 months). • Give 3 month supply.
Sterilisation (Tubal ligation/Vasectomy) <i>Reproductive tubes closed.</i>	Ensure patient understand that this is permanent and cannot be reversed: avoid if patient unsure.	• No return to fertility. • Surgical complication risks	• Permanent. • Refer for procedure. Consent needed

¹Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease.